

PRODUCT LITERATURE

COREDIN PAEDIATRIC LINCTUS

Each 5 ml contains

Diphenhydramine Hydrochloride	7.0 mg
Ammonium Chloride	67.5 mg

Preservative

Methyl Paraben	0.1 %w/v
Propyl Paraben	0.01 %w/v

Description

Red-coloured clear syrup with sweat taste and fruity odour.

Pharmacodynamics Properties

Mechanism of action:

Antihistaminic (H₁-receptor) - Antihistamines used in treatment of allergy act by competing with histamine for H₁-receptor sites on effector cells. They thereby prevent, but do not reverse, responses mediated by histamine alone. Antihistamines antagonize, in varying degrees, most of the pharmacological effects of histamine, including urticarial and pruritus. Also, the anticholinergic of the most antihistamines provide a drying effect on the nasal mucosa.

Antidyskinetic- The actions of diphenhydramine in parkinsonism and in drug-induced dyskinesias appear to be related to a central inhibition of the actions of acetylcholine, which are mediated via muscarinic receptors (anticholinergic action), and to its sedative effects.

Antitussive- Diphenhydramine suppresses the cough reflex by a direct effect on the cough center in the medulla of the brain.

Pharmacokinetics Properties

Absorption:

Well absorbed after oral administration.

Protein binding:

Diphenhydramine 98-99%.

Biotransformation:

Hepatic (cytochrome P-450 system); some renal.

Half-life:

Elimination; Diphenhydramine- 1 to 4 hours

Onset of action:

Most first-generation antihistamines; 15 to 60 minutes.

Time to peak concentration:

Diphenhydramine; 1 to 4 hours

Elimination:

Most of the antihistamines studied (except cetirizine) are excreted as metabolites within 24 hours.

Indication

For the treatment of cough and upper respiratory congestion due to cold and allergy.

Recommended Dose

To be taken 3 or 4 times a day.

Children (2 - 3 Years)	: ½-1 teaspoonful (2.5 ml)
(3 - 6 Years)	: 1 teaspoonful (5 ml)
(6- 12 Years)	: 1-2 teaspoonfuls (5 ml - 10 ml)

Route of Administration

Oral

Contraindications

Risk-benefit should be considered when the following medical problems exist

- Bladder neck obstruction or
- Prostatic hypertrophy, symptomatic or
- Urinary retention, predisposition to (anticholinergic effects may precipitate or aggravate urinary retention)
- Glaucoma, angle-closure, or predisposition to (anticholinergic mydriatic effect resulting in increased intraocular pressure may precipitate an attack of angle-closure glaucoma)

Glaucoma, open-angle (anticholinergic mydriatic effect may cause a slight increase in intraocular pressure; glaucoma therapy may need to be adjusted)

Hypersensitivity to the antihistamine used

Caution is recommended when dimenhydrinate, diphenhydramine, or hydroxyzine is used, since their antiemetic action may impede diagnosis of such conditions as appendicitis and obscure signs of toxicity from overdosage of other drugs.

Warnings and Precautions

When used for treatment of cough and cold:

(a) Not to be used in children less than 2 years of age

(b) To be used with caution and doctor’s/ pharmacist’s advice in children 2 to 6 years of age.

Cross-sensitivity and/or related problems

Patients sensitive to one of the antihistamines may be sensitive to others.

Carcinogenicity/Tumorigenicity/Mutagenicity

Long-term animal studies to evaluate carcinogenic, tumorigenic, or mutagenic potential of most antihistamines have not been performed.

Pediatrics

Use is not recommended in newborn or premature infants because this age group has an increased susceptibility to anticholinergic side effects, such as central nervous system (CNS) excitation, and an increased tendency toward convulsions.

A paradoxical reaction characterized by hyperexcitability may occur in children taking

antihistamines.

Geriatrics

Dizziness, sedation, confusion, and hypotension may be more likely to occur in geriatric patients taking antihistamines.

A paradoxical reaction characterized by hyperexcitability may occur in geriatric patients taking antihistamines.

Geriatric patients are especially susceptible to the anticholinergic side effects, such as dryness of mouth and urinary retention (especially in males), of the antihistamines. If these side effects occur and continue or are severe, medication should probably be discontinued.

Dental

Prolonged use of antihistamines (except cetirizine, desloratadine, or loratadine) may decrease or inhibit salivary flow, thus contributing to the development of caries, periodontal disease, oral candidiasis, and discomfort.

Interactions with Other Medicaments

- Alcohol or
- CNS depression, producing medications, other (concurrent use may potentiate the CNS depressant effects of either these medications or antihistamines; also, concurrent use of maprotiline or tricyclic antidepressants may potentiate the anticholinergic effects of either antihistamines or these medications)

- Anticholinergics or other medications with anticholinergic activity (anticholinergic effects may be potentiated when these medications are used concurrently with antihistamines; patients should be advised to report occurrence of gastrointestinal problems promptly since paralytic ileus may occur with concurrent therapy)

Apomorphine (prior administration of dimenhydrinate, diphenhydramine, doxylamine, or hydroxyzine may decrease the emetic response to Apomorphine in the treatment of poisoning)

- Monoamine oxidase (MAO) inhibitors, including furazolidone and procarbazine (concurrent use of MAO inhibitors with antihistamines may prolong and intensify the anticholinergic and CNS depressant effects of antihistamines; concurrent use is not recommended)

Ototoxic medications (concurrent use with antihistamines may mask the symptoms of ototoxicity such as tinnitus, dizziness, or vertigo)

Photosensitizing medications, other (concurrent use of these medications with antihistamines may cause additive photosensitizing effects)

Pregnancy Reproduction and Lactation

Pregnancy/Reproduction

Pregnancy;

Adequate and well-controlled studies in humans have not been done.

Studies in rats and rabbits at doses up to 5 times the human dose have revealed no evidence of impaired fertility or harm to the fetus.

FDA Pregnancy Category B.

Breast-feeding

First-generation antihistamines may inhibit lactation because of their anticholinergic actions.

Small amounts of antihistamines are distributed into breast milk; use is not recommended in nursing mothers because of the risk of adverse effects, such as unusual excitement or irritability, in infants.

Side Effects

Those indicating need for medical attention

Incidence less frequent or rare

Anaphylaxis (cough; difficulty swallowing; dizziness; fast heartbeat; hives; itching; puffiness or swelling of the eyelids or around the eyes, face, lips or tongue ; shortness of breath; skin rash; tightness in chest; unusual tiredness or weakness ; wheezing)

cardiac arrhythmias/palpitations/tachycardia (fast pounding or irregular heartbeat or pulse); less frequent or rare with azatadine, cetirizine, clemastine, cyproheptadine, desloratadine, dexchlorpheniramine, diphenhydramine, or loratadine

cholestasis, hepatitis or other hepatic function abnormalities (abdominal or stomach pain; chills; clay-colored stools or dark urine; diarrhea; dizziness; fever; headache; itching)

convulsions or seizures,

edema (swelling)

Those indicating need for medical attention only if they continue or are bothersome

Incidence more frequent

Drowsiness

dryness of mouth, nose, or throat

gastrointestinal upset, stomach pain, or nausea ; with azatadine and diphenhydramine

Note: Tolerance to central nervous system effects may develop quickly with some antihistamines, so that sedation is no longer troublesome after a few days.

Incidence of sedation may increase when the recommended doses of loratadine are exceeded.

Incidence less frequent or rare

Blurred vision or any change in vision with azatadine, cetirizine, cyproheptadine, diphenhydramine, and loratadine,

paradoxical reaction (nightmares; unusual excitement, nervousness, restlessness, or irritability) except with azatadine, chlorpheniramine, cyproheptadine, desloratadine, hydroxyzine, and loratadine

thickening of mucus with cyproheptadine, dexchlorpheniramine and diphenhydramine

abnormal coordination (clumsiness or unsteadiness), **constipation, diarrhea, early menses, fatigue, tremor**

Note: *Confusion; difficult or painful urination; drowsiness; dizziness; and dryness of mouth, nose, or throat are more likely to occur in the elderly.*

Nightmares, unusual excitement, nervousness, restlessness, or irritability is more likely to occur in children and elderly patients.

Symptoms and Treatment of Overdose

Symptoms of overdose

Anticholinergic effects (clumsiness or unsteadiness; severe drowsiness; severe dryness of mouth, nose, or throat; flushing or redness of face; shortness of breath or troubled breathing); especially with azatadine and clemastine

cardiac arrhythmias (fast or irregular heartbeat) ; less frequent with azatadine and clemastine

CNS depression (severe drowsiness)

CNS stimulation (hallucinations, seizures, trouble in sleeping)

hypotension (feeling faint)

Note: Anticholinergic and CNS stimulant effects are more likely to occur in children with overdose. Hypotension may also occur in the elderly at usual doses.

Anticholinergic and CNS effects may be less likely to occur with, cetirizine, desloratadine or, loratadine, than with the first-generation antihistamines.

somnolence (sleepiness or unusual drowsiness) especially with desloratadine

Treatment of overdose

Since there is no specific antidote for overdose with antihistamines, treatment is symptomatic and supportive.

To decrease absorption:
Induction of emesis (syrup of ipecac recommended); however, precaution against aspiration is necessary, especially in infants and children.

Gastric lavage (isotonic or 0.45% sodium chloride solution) if patient is unable to vomit within 3 hours of ingestion.

To enhance elimination:
Saline cathartics (milk of magnesia) are sometimes used.

Specific treatment:
Vasopressors to treat hypotension; however, epinephrine should not be used since it may further lower blood pressure.

Oxygen and intravenous fluids.

Precaution against use of stimulants (analeptic agents) because they may cause seizures.

Packing

Packed in plastic bottle of 60 ml and 120 ml.

Pharmaceutical Precautions

Store below 30°C in a dry place, protected from direct light.
Keep away from reach of children.
Jauhkan daripada kanak-kanak

SHAKE WELL BEFORE USE.

Expiry Date

2 years from date of manufacture

Name and Address of Distributor:
ZONTRON PHARMACEUTICALS SDN.BHD. (445695-T)
Lot 10 & 11, PERDA Industrial Park,
Lorong IKS Simpang Ampat B,
14100 Simpang Ampat, S.P.S.,
Pulau Pinang, Malaysia

Name and Address of Product Registration Holder / Manufacturer:
TERAPUTICS SDN. BHD. (590500-W)
Lot 10 & 11, PERDA Industrial Park,
Lorong IKS Simpang Ampat B,
14100 Simpang Ampat, S.P.S.,
Pulau Pinang, Malaysia

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